

Department of Orthopaedics
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 28.0 Yr/M
Admission No.: anonymous	Department Unit: Orthopaedics (Ortho Unit 2)	Consultant/Faculty: anonymous
Ward: 2 A 3 HDU Ward	Room/Bed: 2 A 3 HDU/ 2a3 HDU 2	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 23-Mar-2023 12:57	D.O.D. : 03-Apr-2023	D.O. Print Report: 08-Feb-2026 15:32

FINAL DIAGNOSIS:

ACL TEAR LEFT SIDE

OPERATION PERFORMED:

LEFT ACL RECONSTRUCTION BY ARTHROSCOPY On 31-Mar-2023

SURGEONS: Dr Sandeep Kumar Neema

ASSISTANT SURGEONS/SENIOR RESIDENTS: Dr Arkesh M, DR RUDRA, DR DIKSHANT

OPERATIVE FINDINGS: under all aseptic precaution after effect of anaesthesia left lower limb is painted and draped sterile. curvilinear incision of three centimeters given medial to tibial tuberosity. gracilis and semitendinosus tendon identified. achesia/venecular excised and tendon made free. Tendon harvested with tendon stripper. hexavert graft prepared. standard lateral and inferomedial port made. diagnostic arthroscopy done. ACL found to be torn and adhered to PCL. PCL, MM, LM found to be intact. femoral footprint of ACL identified. femoral tunnel made over guidewire using 7mm posterior offset. Femoral tunnel drilled with 4mm and then 10mm for 30. Tibial foot of ACL identified. Tibial tunnel made over guide wire with 50 degree ACL, tibial tunnel and reaming done. graft passed and endobutton flipped. Tibial side fixed with 10x 35mm, interference and suture disc. cycling done. Wound closed in layers. antiseptic dressing done.

CHIEF COMPLAINTS:

PAIN IN LEFT KNEE DURING JUMPING AND TWISTING FOR 4 MONTHS

HISTORY OF PRESENT ILLNESS: Patient was apparently asymptomatic four months back. When he developed pain in left knee due to injury to left knee while playing cricket on 2 November 2022 At 7:30 am following which patient went to local hospital for primary treatment but pain and deformity persisted. So patient came to AIIMS for further management.

No history of trauma to chest, abdomen and head, no history of loss of consciousness, vomiting

EXAMINATION:

03-Apr-2023



General Physical Examination

Local Examination

LEFT KNEE-

Type

skin intact,no swelling at left
knee,no discharging
sinus,surgical scar,ROM at left
knee normal,pain present at left
knee normal,pain present at left
knee when internally
rotated,anterior drawer test
positive,active toe movement
present,DPA/PTA palpable

Sensorium

Normal

70

Blood Pressure

Systolic

122

Diastolic

70

Temperature (in Fahrenheit)

Afebrile

Clubbing

No

JVP

Normal

70

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
01-Apr-2023 07:13	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.02	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	2.05	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	1.21	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	7.25	2 - 7 x10 ³ /μL
		Basophil	0.1	00 - 01 %
		Eosinophils	0.2	01 - 06 %
		HCT	42.7	40 - 50 %
		Hemoglobin	13.4	13 - 17 g/dl
		Lymphocytes	19.4	20 - 40 %
		MCH	23.3	27 - 32 pg
		MCHC	31.4	31.5 - 34.5 g/dL
		MCV	74.4	83 - 101 fL
		Monocytes	11.5	2 - 10 %
		Neutrophils	68.8	40 - 80 %
		Platelet Count	226	150 - 410 x10 ³ /μL
		RBC	5.74	4.5 - 5.5 x10 ⁶ /μL

24-Mar-2023 01:27	Rft	Red Cell Distribution Width (RDW CV)	14.5	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	10.54	4 - 10 x10 ³ /μL
		Chloride	99	102 - 109 mmol/L
		Creatinine	0.98	0.9 - 1.3 mg/dl
		Potassium	4.18	3.5 - 5 mmol/L
		Sodium	138	135 - 145 mmol/L
		Urea	17	15.0 - 39.0 mg/dl
		APTT HEADER	24.8	23.2 - 32.4 sec
		APTT NOTE	Refer Reports*	-
		clinical details	--	-
24-Mar-2023 01:27	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	BLOOD GROUPING	Blood Group	O
		ABO and RhD	Note	Refer Reports*
			RH Type	Positive
24-Mar-2023 01:27	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.02	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.08	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	3.49	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.50	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	4.13	2 - 7 x10 ³ /μL
		Basophil	0.2	00 - 01 %
		Eosinophils	1.0	01 - 06 %
		HCT	45.3	40 - 50 %
		Hemoglobin	13.9	13 - 17 g/dl
		Lymphocytes	42.5	20 - 40 %
		MCH	23.0	27 - 32 pg
		MCHC	30.7	31.5 - 34.5 g/dL
		MCV	74.9	83 - 101 fL
		Monocytes	6.1	2 - 10 %
		Neutrophils	50.2	40 - 80 %
		Platelet Count	203	150 - 410 x10 ³ /μL
		RBC	6.05	4.5 - 5.5 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	14.4	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	8.22	4 - 10 x10 ³ /μL
		ERYTHROCYTE SEDIMENTATION	ESR	Refer Reports*
				-



RATE (ESR)			
HBsAg (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
HCV Antibody (ELISA)	Interpretation	Non Reactive	-
	Result	Refer Reports*	-
Liver Fuction Test (Lft)	Albumin	4.3	4.0 - 5.0 gm/dL
	ALP	101	50 - 116 U/L
	ALT	31	7 - 41 U/L
	AST	26	12 - 38 U/L
	Direct Bilirubin	0.19	0.1 - 0.4 mg/dl
	globulin	3.0	2.0 - 3.5 gm/dL
	Indirect Bilirubin	0.70	0.20 - 0.90 mg/dl
	Total Bilirubin	0.89	0.3 - 1.3 mg/dl
	Total Protein	7.3	6.7 - 8.6 gm/dL
PROTHROMBIN TIME (PT INR)	clinical details	--	-
	International Normalised Ratio (INR)	1.0	-
	PT INR Header	10.1	09.6 - 12.2 sec
	PT INR Note	Refer Reports*	-
Rft	Chloride	101	102 - 109 mmol/L
	Creatinine	0.98	0.9 - 1.3 mg/dl
	Potassium	4.10	3.5 - 5 mmol/L
	Sodium	137	135 - 145 mmol/L
	Urea	15	15.0 - 39.0 mg/dl

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: TREATMENT GIVEN-

INJ. CEFUROXIME 1.5 GM IV BD
 INJ. AMIKACIN 500MG IV BD
 INJ. PCM 1 GM IV TDS
 INJ. PANTOPRAZOLE 40 MG IV OD
 INJ. ONDANSETRON 4MG IV BD
 INJ. DICLOFENAC 75 MG IN 100ML NS IV BD
 INJ. TRAMADALOL 100MG IN 100ML NS IV SOS
 TAB MVBC OD
 TAB CA2 VitD3 OD
 Tab Vit C 500 mg TDS

CONDITION AT DISCHARGE: PT IS HEMODYNAMICALLY STABLE.

ADVICE ON DISCHARGE:

TAB PCM 650MG SOS
 TAB PANTOPRAZOLE 40MG OD X 5 DAYS



C-DAC: Centre for Development of Advanced Computing
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91-0120-3063311-13.



NON WEIGHT BEARING
ACTIVE TOE MOVEMENT
HIP ANKLE ROM EXERCISES
ACTIVE TOE MOVEMENT

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DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 13-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW IN OPD ON MON/THURSDAY

Summary Prepared By

anonymous

Summary Approved By

anonymous